Community Health Worker (CHW) Sustainability and allied professions in Arizona

Alicia Hernandez[1,2], Binoli Herath[1], Mindy McEntee, PhD[1]

[1]College of Health Solutions, Arizona State University; [2]Mel and Enid Zuckerman College of Public Health, University of Arizona

INTRODUCTION

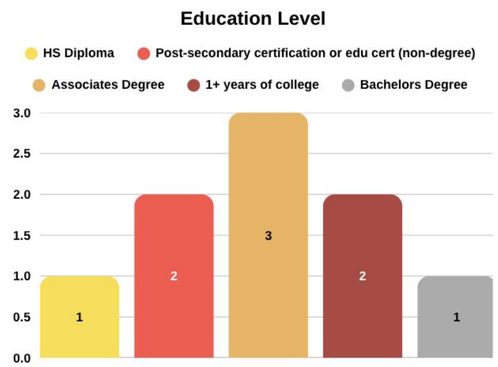
Community Health Workers (CHWs), including promotores, community-based health workers (CBHWs), peer recovery specialists, and community health representatives (CHRs), are frontline public health workers who serve as trusted liaisons between healthcare systems and the communities they serve. CHWs play an important role in improving access to care, providing culturally responsive health education, supporting chronic disease management, and addressing social determinants of health[1,3]. As healthcare systems increasingly prioritize health equity and value-based care, there has been growing attention toward formalizing CHW roles through certification programs, workforce development initiatives, and Medicaid reimbursement. Despite the demonstrated value of CHWs in improving health outcomes and reducing disparities, the workforce continues to face structural challenges such as low wages, job insecurity, inconsistent training requirements, and limited reimbursement pathways[2].

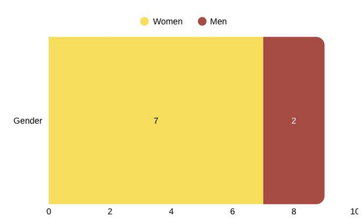
AIM

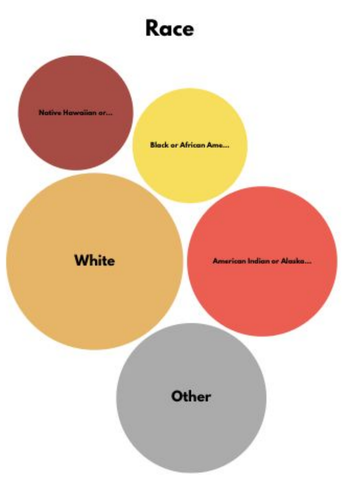
The research question for this project asks how trainees in the Arizona State University Community Health Worker Training Program who previously worked as Behavioral Health Technicians (BHTs), paraprofessionals, or Peer Recovery Support Specialists (PRSS) perceive the role of CHWs compared to their previous roles. The objective of this study is to define the roles of CHWs, BHTs, and PRSS in order to identify overlapping responsibilities and professional boundaries. In addition, the study examines participants' perspectives on training and workforce experiences when individuals are cross-trained in these roles. Findings from this study aim to inform workforce development efforts and support clearer role definitions within the community health workforce.

METHODS

Structured qualitative interviews were conducted with trainees enrolled in the ASU Community Health Worker Training Program. Participation was voluntary and recruitment focused on trainees who reported prior experience working as BHTs, PRSS, or other behavioral health paraprofessionals in their initial application. Interviews explored participants' perceptions of the workforce, including job market experiences, role responsibilities, career motivations, training experiences, and professional opportunities. Interviews were conducted via Zoom, audio-recorded, and transcribed verbatim. The transcripts were analyzed using thematic analysis to identify common patterns and experiences across participants. Interviews and coding were conducted by staff within the CHW Training Program.







RESULTS

Data collection is ongoing, with nine participants interviewed to date. Participants ranged in age from 25 to 62 years and included seven females. Participants identified as White (n=2), Hispanic (n=4), Black (n=2), and Other (n=1). Preliminary findings indicate..

- Participants view their roles as collaborative with related professions but report significant gaps in role clarity and recognition within healthcare settings.
- Educational and training requirements varied by position; however, many participants perceived the qualifications for CHWs, BHTs, and PRSS to be comparable across roles.
- Participants frequently described CHWs as occupying flexible and multifaceted positions within healthcare and community systems.
- CHWs were commonly described as liaisons between communities and healthcare institutions, with responsibilities that extend beyond traditional care coordination to include outreach, advocacy, and social support.
- Despite this flexibility, role ambiguity often led participants to navigate unclear professional boundaries or take on responsibilities outside their defined roles.

Participants also reported workforce challenges, including limited recognition from medical staff, instability due to grant-based funding, and expectations to perform multiple roles interchangeably. These conditions contributed to frustration, burnout, and concerns about long-term workforce sustainability. Compensation was frequently described as inadequate relative to the responsibilities required for the position, with participants suggesting that a wage of $20–$25 per hour would better reflect the demands of the work. Pay differences were influenced by education level, organizational setting, and geographic location, and several participants noted discomfort initiating conversations about compensation with employers.

Theme	Subthemes	Participants (n=9)	Example Quote
Organizational Structure and Role Hierarchy	Role clarity, hierarchy, collaboration with behavioral health specialists	6	" I think in a CHW you can work in a wider variety of, you know, roles."
Training and Professional Preparation	Classroom training, role-playing exercises, skill development	5	"Trainings were mostly connected to my job. Majority of them were about a week long...we were literally role-playing scenarios."
Community Engagement and Client Relationships	Trust-building, resource navigation, patient advocacy	4	"you're still providing help. You're still listening to their concerns. You're still addressing concerns. There's still providing a resource or a service"
Workforce Sustainability and Compensation	Burnout, financial stability, compensation concerns	4	"It's poor. I'll be honest, it's very poor compensation.... and the burnout is so bad."
Personal Motivation and Commitment to Service	Desire to help others, personal experience, career transitions	3	"I'm going to continue doing what I'm doing. I definitely feel that i definitely feel this is something that I can do and make a difference in people's lives."

4 out of 9 interviewees mentioned doing this to help people and as a new role

CONCLUSIONS

Participants described considerable variability in how CHWs are positioned within healthcare organizations, including differences in authority, recognition, and collaboration with behavioral health and peer support professionals. Organizational barriers and unclear role definitions may limit the effective integration of CHWs into multidisciplinary care teams. These findings suggest the need for clearer role delineation, standardized training pathways, and stronger institutional recognition of CHWs within healthcare systems. Improving workforce sustainability may also require expanding reimbursement mechanisms, strengthening career advancement pathways, and establishing fair and consistent compensation structures. Addressing these workforce challenges could enhance the stability of the CHW workforce and support more effective integration of CHWs into healthcare and public health systems, ultimately improving community health outcomes.

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College of Health Solutions
Arizona State University

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